

- If they feel "wiped out" the day after exercising, walk slower or for a shorter distance until they can work up to a better pace.
- They should get enough rest at night. Sleeping with 1 or 2 pillows under their upper body and head may help breathe easier.

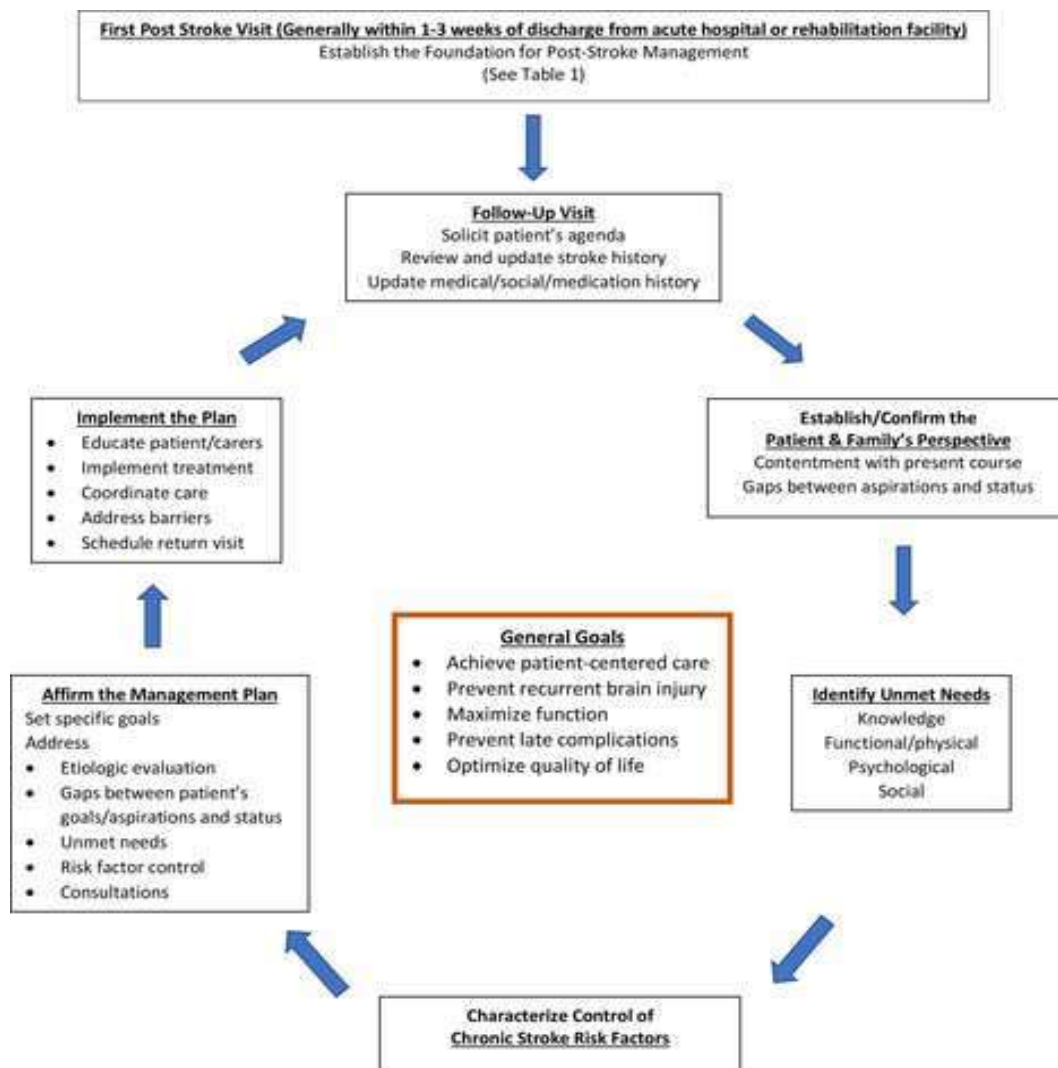
6. Lifestyle changes.

- Do not smoke. Smoking can make a heart condition worse. If they need help quitting, talk to the doctor about stop-smoking programs and medicines. These can increase chances of quitting for good. Quitting smoking may be the most important step to take to protect the heart.
- Limit alcohol. Too much alcohol can cause health problems.
- Avoid getting sick from colds and influenza (flu). The patient should get a pneumococcal vaccine shot. If they have had one before, ask the doctor whether they need a second dose. Get a flu shot each year. If they must be around people with colds or the flu, encourage them to wash hands often.

Stroke

The care of patients with stroke begins in the hospital and continues in the community, where recovery, reintegration, and health maintenance take place over the years that follow. Primary care clinicians provide most of this long-term care. The needs of these patients can be complex according to the temporal phase of their illness, the cause and severity of their stroke, and other factors, including the presence of other chronic health conditions.⁷ Approximately 50% to 80% will have hypertension, 20% to 30% will have diabetes, and 10% to 30% will have comorbid heart disease or atrial fibrillation. Lung disease, depression, anxiety, kidney disease, and arthritis are also common.

Overall Strategy



Post-stroke care is an iterative process of *assessment, management, and feedback* that adapts over time to the changing needs of the patient. To operationalize this process, there's need for primary care visits that is based on current notions of chronic care management to help clinicians achieve generally accepted goals for post-stroke care, that is: *provide patient-centered care, prevent recurrent brain injury, maximize function, prevent late complications, and optimize quality of life*. The first of these, patient-centered care, is a philosophy that guides clinicians toward health outcomes that are most valued by individual patients. It emphasizes *the alleviation of pain, fear, and anxiety*. It emphasizes patient autonomy and communication to identify patient values and preferences that underlie autonomous decisions.

Fundamentals of caregiving Post-stroke

Tips that can help a caregiver handle a patient after stroke.

- **It's better to find out than miss out.** Be aware of the patient's medications and their side effects. Find out if your home should be modified to meet the needs of the stroke survivor. Ask a doctor, nurse or therapist to answer your questions about what to expect.
- **Reduce risks,** or stroke may strike again. Survivors are at high risk of having another stroke. Make sure your loved one eats a healthy diet, exercises, takes medications as prescribed and visits their healthcare provider regularly.
- **Many factors influence recovery:** where in the brain the stroke occurred; how much of the brain was affected, the survivor's motivation; caregiver support; the quantity and quality of rehabilitation; and the survivor's health before the stroke.
- **Gains can happen quickly** or over time. The most rapid recovery usually occurs during the first three to four months after a stroke, but some survivors continue to recover well into the first and second year after their stroke.
- **Some signs point to physical therapy.** Caregivers should consider assistance from a physical or occupational therapist if their loved one has: *dizziness; imbalance* that results in falls; difficulty walking or moving around daily; inability to walk six minutes without stopping to rest; inability to participate in or complete daily activities.
- **Don't ignore falls.** Falls after stroke are common. If a fall is serious and results in severe pain, bruising or bleeding, take your loved one to the emergency room. If your loved one has minor falls more than two times within six months, see your physician or physical therapist for treatment.
- **Measuring progress matters.** How much acute rehabilitation therapy your loved one receives depends partly on the rate of improvement. Survivors in acute rehabilitation are expected to make measurable functional gains every week based on the Functional Independence Measure Score (FIMS). Functional improvements include daily, mobility and communication skills. The typical rehabilitation expectation is improving 1-2 FIMS points per day.
- **A change in abilities can result in a change in services.** Medicare coverage for rehabilitation therapies may be available if your loved one's physical function has

changed. Any improvement or decline in motor skills, speech or self-care since the last time your loved one was in therapy may mean eligibility changes in services.

- **Monitor changes in attitude and behavior.** Evaluate whether your loved one is having a hard time controlling emotions. Consult a healthcare provider to develop a plan of action.
- **Stop depression before it hinders recovery.** Post-stroke depression is common, with as many as 30-50 percent of stroke survivors depressed in the early or later phases of post-stroke. Post-stroke depression can significantly affect your loved one's recovery and rehabilitation. Consult a healthcare provider to develop a plan of action.

DIABETES

Diabetes is a chronic (long-lasting) health condition that affects how your body turns food into energy. Your body breaks down most of the food you eat into sugar (glucose) and releases it into your bloodstream. When your blood sugar goes up, it signals your pancreas to release insulin. Insulin acts like a key to let the blood sugar into your body's cells for use as energy.

With diabetes, your body doesn't make enough insulin or can't use it as well as it should. When there isn't enough insulin or cells stop responding to insulin, too much blood sugar stays in your bloodstream. Over time, that can cause serious health problems, such as heart disease, vision loss, and kidney disease.

There isn't a cure yet for diabetes, but losing weight, eating healthy food, and being active can really help. Other things you can do to help:

- Take medicine as prescribed.
- Get diabetes self-management education and support.
- Make and keep health care appointments.

Types of Diabetes

There are three main types of diabetes: type 1, type 2, and gestational diabetes (diabetes while pregnant).